

for individuals are for medical care and higher education. The former is part of the CPI bundle and is separately tabulated by the Bureau of Labor Statistics; clearly the elderly and sick consume more medical care than the average consumer. The latter is measured by the Higher Education Price Index and is distributed annually by Commonfund, a nonprofit institution specializing in asset management for endowments and foundations. (Higher education expenses do enter the CPI bundle, but I use the Higher Education Price Index for this analysis as it is more relevant for university endowments.) Figure 11.2 plots both series, together with CPI, from 1984 to 2008.

Figure 11.2 shows that both medical care costs and higher education expenses have been increasing faster than the overall rate of inflation as measured by the vanilla CPI. From 1984 to 2008, the average annual CPI increase has been 3.1% while the average inflation rate for medical care and higher education has been 5.3% and 4.0%, respectively. Due to the effect of compound interest, a 1% difference over twenty to thirty years can really kill you (literally, in the case of inadequate medical care). A basket of \$10,000 worth of general goods and services at the beginning of 1984 would cost \$21,600 at the end of 2008, but \$10,000 worth of medical care in 1984 would cost \$36,100 in 2008.

The difference between the CPI and an investor's own consumption basket is called *basis risk*, and Figure 11.2 shows that it can be substantial. There are ways to hedge against basis risk, but the most important is to first hedge against a general rise in all prices. Then an investor can tilt her portfolio to assets that may have higher exposures to medical care risk, for example, if her liabilities or

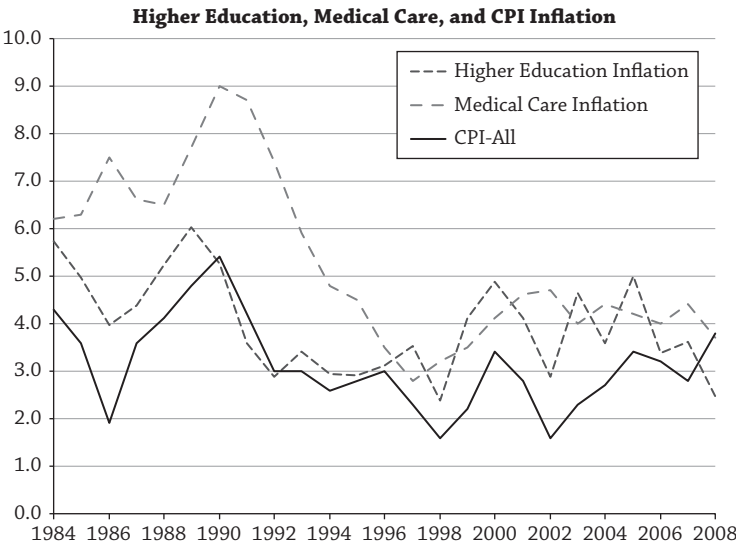


Figure 11.2